ADR Reporting & Documentation

Task 2: Completing a structured ADR reporting form for a case scenario

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About this task

This task completes a structured Adverse Drug Reaction (ADR) report for a simulated case, following the **Suspected Adverse Drug Reaction Reporting Form** of the Pharmacovigilance Programme of India (PvPI), CDSCO. The aim is to document the case accurately and completely so it can be assessed and added to the national drug safety database.

1. Identify	2. Document	3. Submit	4. Analyse
Recognise a suspected ADR	Fill in the reporting form	Send to an ADR Monitoring Centre	Causality assessment and signal detection

Case scenario

A 38-year-old woman visited a dental clinic with sudden, sharp, electric-shock-like pain over the right lower jaw, triggered by brushing and chewing. Dental examination found no dental cause. She was referred to a neurologist, who diagnosed **right trigeminal neuralgia** and started **carbamazepine**.

About three weeks later she developed fever, sore throat and burning eyes, followed two days later by a painful red rash with blisters on the face and trunk, painful erosions in the mouth and red eyes. She was admitted to hospital and diagnosed with **Stevens-Johnson syndrome (SJS)**. Carbamazepine was stopped immediately.

Suspected Adverse Drug Reaction Reporting Form

Format based on the PvPI / CDSCO reporting form

A. PATIENT INFORMATION

Patient initials	R.K.
Age at time of event	38 years
Gender	Female
Weight	58 kg

B. SUSPECTED ADVERSE REACTION

Date of reaction onset	24 Sep 2026 (prodrome); rash from 26 Sep 2026
Date of recovery	Not yet recovered (recovering at time of report)
Description of reaction	Fever, sore throat and burning eyes, followed by a painful erythematous rash with blisters on face and trunk, oral mucosal erosions and bilateral conjunctivitis. Skin detachment less than 10% of body surface area. Diagnosed as Stevens-Johnson syndrome.

C. SUSPECTED MEDICATION(S)

Name (brand / generic)	Carbamazepine tablets
Manufacturer	Not known
Batch / lot number	Not available
Expiry date	Not available
Dose	100 mg twice daily, increased to 200 mg twice daily on 12 Sep 2026
Route / frequency	Oral / twice daily
Therapy start date	05 Sep 2026
Therapy stop date	26 Sep 2026
Reason for use	Right trigeminal neuralgia

Action taken after reaction

- [x] **Drug withdrawn**
- [ ] Dose reduced
- [ ] Dose increased
- [ ] Dose not changed
- [ ] Not applicable
- [ ] Unknown

Reaction reappeared after reintroduction?

- [ ] Yes
- [ ] No
- [ ] Effect unknown
- [x] **Not reintroduced (contraindicated after SJS)**

CONCOMITANT MEDICAL PRODUCTS

Name	Dose / route / frequency	Start date	Stop date	Reason
Paracetamol	500 mg oral, when needed	Used occasionally for years	Continued	Headache

No herbal remedies or other self-medication reported.

RELEVANT TESTS / LABORATORY DATA

Complete blood count	Mild leucocytosis
Liver and kidney function	Within normal limits
Infection screen	Mycoplasma pneumoniae and herpes simplex: negative
Skin biopsy	Full-thickness epidermal necrosis, consistent with SJS
HLA-B*15:02	Positive (tested after the event)

OTHER RELEVANT HISTORY

No known drug allergy. No liver or kidney disease. No previous similar reaction. Non-smoker; no alcohol use. Not pregnant.

SERIOUSNESS OF THE REACTION

- ☐ Death
- ☐ Life threatening
- ☑ **Hospitalisation (initial / prolonged)**
- ☐ Disability
- ☐ Congenital anomaly
- ☐ Required intervention to prevent permanent damage
- ☐ Other medically important

OUTCOME

- ☐ Recovered
- ☑ **Recovering**
- ☐ Not recovered
- ☐ Fatal
- ☐ Recovered with sequelae
- ☐ Unknown

D. REPORTER DETAILS

Name	Dr. Zainab Umar
Profession / speciality	Dental Surgeon (BDS)
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Date of this report	12 Oct 2026
Signature	Dr. Zainab Umar

Case narrative

A 38-year-old female (R.K.) with right trigeminal neuralgia was started on oral carbamazepine 100 mg twice daily on 05 Sep 2026, increased to 200 mg twice daily on 12 Sep 2026. On 24 Sep 2026 (day 20 of therapy) she developed fever, sore throat and burning eyes. On 26 Sep 2026 she developed a painful erythematous blistering rash on the face and trunk with oral erosions and bilateral conjunctivitis, involving less than 10% of body surface area. She was hospitalised and diagnosed with Stevens-Johnson syndrome, confirmed by skin biopsy. Carbamazepine was withdrawn the same day, and she received supportive care including fluids, wound care, eye care and pain relief. Infective causes were excluded. Her only other medicine, paracetamol, had been used occasionally for years without problems. The lesions began to heal within a week of stopping the drug, and she was recovering at the time of reporting. HLA-B*15:02 testing, done after the event, was positive. Carbamazepine will not be reintroduced, and she has been advised to avoid carbamazepine and related aromatic anticonvulsants.

Causality assessment

WHO-UMC

Criterion	Finding
Time relationship	Onset 20 days after starting; typical for SJS (usually 4 to 28 days)
Other explanations	Unlikely; infections excluded, paracetamol long tolerated
Response to withdrawal	Improved after stopping
Rechallenge	Not done (contraindicated)
Known reaction	SJS is well documented with carbamazepine, especially with HLA-B*15:02

Result: Probable / Likely

Naranjo scale

Question	Answer	Score
1. Previous conclusive reports?	Yes	+1
2. Appeared after the drug was given?	Yes	+2
3. Improved when the drug was stopped?	Yes	+1
4. Reappeared when the drug was given again?	Not done	0
5. Alternative causes?	No	+2
6. Reappeared with placebo?	Do not know	0
7. Drug in toxic blood concentration?	Do not know	0
8. Dose-related?	Do not know	0
9. Similar reaction to same drug before?	No	0
10. Confirmed by objective evidence?	Yes (skin biopsy)	+1
Total		**7 (Probable)**

Conclusion

Suspected drug: Carbamazepine
Reaction: Stevens-Johnson syndrome
Seriousness: Serious (hospitalisation)
Causality: WHO-UMC Probable; Naranjo 7 (Probable)
Action: Drug withdrawn permanently; patient advised to avoid carbamazepine and related drugs.

Where this report is submitted

In India, ADR reports are submitted to the nearest **ADR Monitoring Centre (AMC)** under PvPI, or directly through the **PvPI toll-free helpline (1800-180-3024)** or the **ADR PvPI mobile app**. Serious reactions such as this one should be reported as early as possible.

Key learning points

- A valid report needs four minimum items: an identifiable patient, a suspected drug, a reaction and an identifiable reporter.
- Accurate dates (start, stop and onset) are essential for assessing causality.
- Concomitant medicines and test results help rule out other causes.
- Seriousness and outcome must be clearly marked so serious cases are prioritised.
- Anyone in healthcare, including dentists, can and should report suspected ADRs.

Note: this case is hypothetical and was written for learning purposes.